

AdvocateAuroraHealth

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I. PURPOSE

The system Security and Public Safety Department establishes and maintains physical controls for AAH sites. This policy establishes security as the focal point for physical security assessment and guidelines. This policy identifies the roles of the Security and Public Safety (Security), Design and Construction, Facilities, and Healthcare Information Technology (HIT) Departments. These are the primary departments that collaborate to continually assess and improve the physical security of legacy sites and design new sites according to laws, regulations, regulatory guidance, and organizational guidelines. It summarizes the approach to physical security guidelines in a system with both legacy and newly constructed sites; and identifies the responsibilities of contributing departments to maintain an appropriate physical security posture. This policy and procedure also defines the parameters for allowing access to AAH occupied facilities and restricted areas within facilities.

II. SCOPE

This policy and procedure applies to Advocate Aurora Health, Inc. and any entity or facility owned and controlled by Advocate Aurora Health.

III. DEFINITIONS/ABBREVIATIONS

- A. **Advocate Aurora Sites** – Any building, hospital, support center, office, warehouse, data center, etc. that is owned, leased or rented by an Advocate Aurora Health Care entity. Parking lots and structures are generally not considered for the purpose of access control.
- B. **Security Sensitive Areas (SSA)** – A location whose function or activity presents an environment in which there is significant potential for injury,

abduction, or security loss that would most likely severely impact the ability of the organization to render a high quality of patient care, and the mere existence of that area or function is in and of itself, a primary risk factor. SSAs should be identified during the security risk assessment process. Common security sensitive areas for health care facilities include emergency departments, pharmacies, mental health units, infant and pediatric areas, laboratories, specialized treatment clinics (e.g. substance abuse, abortion), surgery, intensive care units, IT closets and data centers and plant services. Sensitive areas can also include areas of business sensitivity (i.e. patient records storage).

- C. **Support Person** - A family member, companion, friend or other individual who is there to support the patient during the course of their hospital stay.
- D. **Design Guidelines for Healthcare Facilities** – International Association of Healthcare Security and Safety (IAHSS) industry guidelines that provide guidance to healthcare security practitioners, architects, and others involved in the design process in order to ensure that these best practices are considered and integrated, where possible, into each new and renovated space.
- E. **Det Norske Veritas /National Integrated Accreditation for Healthcare Organizations (DNV NIAHO)**
 - the applicable quality management program based upon the Centers for Medicare and Medicaid Services Conditions of Participation 42 CFR § 482 and State Operations Manual Regulations and Interpretive Guidelines for Hospitals.
- F. **Security Command Center** – 24-hour staffed and operated locations that provide centralized, dedicated security system monitoring, response, and dispatch functions.
- G. **Security Vulnerability Assessments** - SVAs
- H. **Advocate Aurora Enterprise Security Guidelines and Standards for AMG Medical Offices and Advocate Health Care Support Centers** – The design guidelines for the construction of new non-acute clinical sites in the Advocate Aurora system.
 - A sampling of types and sizes of facilities was used in a threat assessment to determine an efficient way to design consistent quality without performing an individual risk assessment every time.
 - The guidelines allow variation due to site-specific characteristics/limitations. They prescribe the use of existing or conforming security systems and appropriate technical configuration so that they can be integrated into the centralized Security Command Center for monitoring and response. It also includes standards for technical specifications of security system components. The guidelines were developed for Advocate before the merge and are now being adapted to incorporate new Aurora sites as well.

IV. POLICY

- A. The size, complexity, and scope of services provided within each facility vary. Security design will be aligned for the environment and function. Physical security design should include:
 - 1. Supporting patient care,
 - 2. Providing a positive employee and consumer experience,
 - 3. Mitigating risk, and;
 - 4. Addressing real and perceived security concerns.
- B. New construction and renovation must incorporate the most current physical security guidelines and best practices. For legacy sites, this will be completed as resources become available and as legacy sites are scheduled for improvements, based on risk and funding availability.
- C. Security system improvements must use or conform with existing platforms. When possible, they should be integrated into the centralized Security Command Center for monitoring and response.
- D. The Public Safety and Security Department centrally manages all physical security related systems and operations for AAH owned and/or operated facilities. Public Safety and Security manages parking and valet operations where provided.
- E. Public Safety and Security should balance the operational need for access with best practice security measures. The identity of physicians and team members must be visible. .
 - 1. Team Members & Physicians: Team members, physicians, and those working in a team member-role (e.g. volunteers, long term contractors, etc.) are required to wear a photo identification badge at all times while on duty and in AAH facilities. The photo identification badge will be worn on the outermost garment between the shoulder and the waist.
 - 2. Patients: All patients will be identified in accordance with existing AAH Patient Identification procedures.
 - 3. Independent Contractors: Contractors will be issued a contractor badge upon completion of required safety training.
 - 4. Hospital Visitors: Each site should establish and publish normal visiting hours. After hours visitation is discouraged and, if necessary, should be coordinated with the unit nursing staff. The patient's support person may visit outside normal visiting hours.

5. Visitation to Sensitive Areas: Visitation to Sensitive Areas will be controlled.
6. AMG visitors: No check-in or badges are required.
- F. Physical Security must be achieved by adhering to policies, procedures, public safety operations, security technology, awareness training for team members, public safety education programs, emergency preparedness, collaboration with other departments, and the use of contract security personnel.
- G. All security and public safety team members will be managed within the Public Safety Department. Team Members will be provided appropriate training and equipment for their duty assignment and location.
- H. Each facility will have a responsible Security or Public Safety leader.
- I. A system-level multidisciplinary group must provide guidance, address concerns, and collaborate with Executive leadership on security matters. This group shall be convened by the Vice President for Security and Public Safety on an as-needed basis and consist of representatives of the following:
 1. AAH Legal
 2. AAH Corporate Compliance
 3. AAH Risk Management
 4. AAH Public Affairs and Marketing
 5. AAH Human Resources
 6. AAH Healthcare Information Technology
- J. The selection, procurement, installation, alteration, and operation of physical security systems and applications in both existing and new construction must be coordinated through the Security and Public Safety Department to ensure compatibility with the master plan. The AAH Security Command Center (ASCC) will monitor security systems. Exceptions to ASCC monitoring must be coordinated through the Security and Public Safety Department.
- K. Mechanical keying and key control are managed by the Facilities and Maintenance Department.
- L. Site changes that could impact physical security effectiveness or operations must be coordinated with the Security and Public Safety Department, such as site/facility closures, department relocations, emergency plans, safety program, wayfinding improvements, policies, physical configuration, or access changes, etc.
- M. A Service Level Agreement details the functions and scope of the Security and Public Safety Department.

V. PROCEDURE

A. Facility Procedure

1. New Hospitals:

- a. Design and Construction, Security, HIT, Facilities, and operational departments collaborate in the design process to achieve optimal physical and technical security in accordance with regulatory and industry standards and guidelines.
- b. Security systems in new hospitals should use or conform with existing security platforms and integrate into an AAH Security Command Center for monitoring, response, and dispatch functions.
- c. When regulations or AAH guidelines do not exist or apply, the IAHS security design guidelines should be applied.

2. Legacy Hospitals:

- a. Legacy hospitals and medical centers undergo annual SVAs in accordance with DNV NIAHO requirements. The SVAs monitor and identify vulnerabilities and prioritize areas of concern in SSAs that require remediation.
- b. Regulations, laws, capital improvement initiatives, significant incidents, or industry standards should be rationale to bring forward operational, physical, and/or technical improvements.
- c. When regulations or AAH guidelines do not exist or apply, the IAHS security design guidelines should be applied.

B. Non-Acute Clinics and Sites of Care Standards

1. New Clinics:

- a. A collective risk assessment was used to develop best practice guidelines for new clinic sites based on their relative size and purpose. The "Advocate Enterprise Security Guidelines Standard Rev 6-8-2018 for AMG Medical Offices and Advocate Health Care Support Centers" are the design guidelines for the construction of new non -acute clinical sites. Guidelines allow for variation due to site-specific characteristics/limitations. They also ensure that the site will use existing or conforming security systems and technical configuration and that they can be integrated into the centralized Security Command Center for monitoring and response.

2. Legacy Clinics:

- a. Legacy clinics, laboratories, and surgical centers have a wide range of physical and technical security configurations. It is not feasible to immediately bring all legacy sites into compliance with the current guidelines (the Advocate Enterprise Security Guidelines Standard Rev 6-8-2018 for AMG Medical Offices and Advocate Health Care Support Centers). Improvements to bring sites into compliance are undertaken as resources become available.
- b. Regulations, laws, capital improvement initiatives, significant incidents, or industry concerns may also spur operational, physical, and/or technical improvements.
- c. Security system improvements must use or conform with existing platforms and should be integrated into the centralized Security Command Center for monitoring and response.

C. Support Center and Business Office Standards

- 1. New Support Centers and Business Offices:
 - a. A collective risk assessment identified best practice guidelines for new support sites based on their relative size and function. The “Advocate Enterprise Security Guidelines Standard Rev 6-8-2018 for AMG Medical Offices and Advocate Health Care Support Centers” are the design guidelines for the construction of new nonacute clinical sites. The guidelines allow for variation due to site specific characteristics/limitations. They also ensure that the site can be integrated into the centralized Security Command Center for monitoring and response.
- 2. Legacy Support Centers and Business Offices:
 - a. Legacy support center and business offices have a wide range of physical and technical security configurations. Improvements to bring sites into compliance with current guidelines (the Advocate Enterprise Security Guidelines Standard Rev 6-8-2018 for AMG Medical Offices and Advocate Health Care Support Centers) are undertaken as resources become available.
 - b. Capital improvement initiatives, significant incidents, laws, regulations, or industry concerns may also spur operational, physical, and/or technical improvements.
 - c. Security system improvements must use or conform with existing platforms and should be integrated into the centralized Security Command Center for monitoring and response.

Roles and Responsibilities

- A. Security and Public Safety should:
 - 1. Use regulations, the IAHS guidelines, and input from other Departments and operational units, to establish and maintain AAH physical security guidelines. The IAHS Design Guidelines for Healthcare Facilities should serve as the default when specific AAH guidelines have not been published.
 - 2. Collaborate with operational departments (e.g. Pharmacy, HIT) to facilitate conformance with any industry-specific regulations, laws, regulatory guidelines, and physical security guidelines.
 - 3. Strive to bring legacy sites into compliance with current regulations and physical security guidelines whenever possible.
 - 4. Conduct security vulnerability assessments to monitor compliance and identify and prioritize improvement opportunities.
 - 5. When physical and technical improvements are not possible or will be delayed, work with units to mitigate risk through operational means.
- B. The Design and Construction Department should coordinate with Security to meet regulations and physical security guidelines.
 - 1. New construction projects should comply with regulations and physical security design guidelines. Include Security, HIT, and other appropriate team members in the design process.
 - 2. Renovation projects should bring the renovated space into compliance with security design guidelines whenever practicable. If guidelines cannot be met, coordinate with Security and HIT to appropriately mitigate risk.
 - 3. Collaborate with operational departments (e.g. Pharmacy) to facilitate conformance with industry-specific regulations, laws, regulatory guidelines, and physical security guidelines.
- C. Facilities should:
 - 1. Incorporate Security into the repair and maintenance process. Alert Security of facility closures, department moves or other activity that has the potential to impact physical security posture and coordinate improvements or repairs.
 - 2. Strive to bring existing sites into compliance with physical security guidelines (e.g. lighting, door hardware).
 - 3. Facilitate conformance with regulations, laws, regulatory guidelines, and physical security guidelines.
- D. HIT should:
 - 1. Identify security measures for HIT-specific Security Sensitive Areas (e.g. data centers).
 - 2. Participate in the design and construction process for both new construction and renovation projects.

3. Facilitate conformance with regulations, laws, regulatory guidelines, and physical security guidelines.

VI. CROSS REFERENCES

- A. AAH Patient Identification
- B. AAH Enterprise Information Security & Chief Information Security Officer Standard
- C. AAH Integrated Desktop/Laptop Security
- D. AAH Network Security
- E. AAH Server Security Integrated

VII. RESOURCES AND REFERENCES

- A. National Integrated Accreditation for Healthcare Organizations (NIAHO) Accreditation Requirements
- B. The HIPAA Security Rule 45 CFR Security Part 164
- C. International Association of Healthcare Security and Safety (IAHSS) Design Guidelines for Healthcare Facilities
- D. American Society for Industrial Security (ASIS) Facilities Physical Security Measures
- E. NIST CSF v1.1 PR-AC-2
- F. NIST SP 800-53 Rev 4 PE-2, PE-3, PE-4, PE-6, PE-9
- G. Centers for Internet Security (CIS) Control 1, 2,12,13,17, 19, 20

VIII. ATTACHMENTS

- A. Advocate Enterprise Security Guidelines Standard Rev 6-8-2018 for AMG Medical Offices and Advocate Health Care Support Centers (IL Only)